

Group Research Proposal — Assignment Brief (Group Product 1)

Foundations of Clinical Research — Level 1: From Clinical Question to Publication

Organisation: Neudata — #ClearDataClearImpact

Assessment type: Group assignment, built in class across Sessions 1–7 and presented in Session 8

Groups: 5 groups of about 6 people, mixed professions, fixed for the whole course

Output: one written research proposal with a publication plan (about 6–8 pages) + one 6-minute presentation (6 slides) in Session 8

Marked with: [Proposal_Rubric.pdf](#) (formative feedback)

The other group product: the draft manuscript from the course dataset — see [Manuscript_Brief.pdf](#)

1. Purpose

Most clinical research starts as a question asked at the bedside. This assignment turns one such question from **your own unit** into a realistic, ethical and feasible research proposal — the kind you could take to your research office, an ethics committee or a small-grant panel.

You will not collect data or analyse anything for the proposal. The goal of this product is to **plan a study well** — a clear question, the right design, good data, honest attention to bias and ethics, a realistic timeline — and to **plan from the start how it will be published**: target journal, reporting guideline, authors and a draft abstract. Each session ends with a proposal-builder slot that adds to one part, so by Session 7 your group has a complete proposal and publication plan, and in Session 8 you present it to the department.

This is the first of the course's **two group products**. The second — a draft manuscript written from the simulated course dataset FLUID-ICU — is built in Sessions 5–8 and described in [Manuscript_Brief.pdf](#). The two products are deliberately different: the proposal is about **your own question** (planning a study that has not been done); the manuscript is about **someone else's data** (reporting a study that has been done).

Throughout the course the instructors use one running example — *In adults with septic shock in the ICU, does a restrictive fluid strategy, compared with usual care, reduce 28-day mortality?* — to show what each part looks like. **Your group chooses its own question.** It may be from any area of practice (for example pressure injuries, hand hygiene, delirium, medication errors, ventilator-associated pneumonia, door-to-needle time). Please do not reuse the running example.

A good topic is: a problem you see often, that matters to patients, that your unit could realistically study in 12–24 months with its own patients and records.

2. What to produce — Part by Part

Use the Word template [Course/Templates/Research_Proposal_Template.docx](#). Its headings match the four Parts below. The template is about 6–8 pages when filled. Write in short, plain sentences; tables and bullet points are welcome. Delete the grey guidance text before submitting.

Part 1 — Built in Sessions 1–2 (Ask; Find and read)

Section	What we expect
Working title	Built from your PICO: names the population, intervention/exposure, outcome and (ideally) the design, e.g. "Restrictive fluids and 28-day mortality in septic shock: a randomised trial". You will refine it in Part 4.
Problem statement & rationale	What is the problem, how big is it, why does it matter here, what is not yet known (½ page).
PICO	Population, Intervention/exposure, Comparison, Outcome (and Time) — one line each.
Research question	One sentence built from the PICO.
FINER check	One line each: Feasible, Interesting, Novel, Ethical, Relevant — and what you changed because of it.
Aim, objectives, hypothesis	One broad aim; 2–3 specific, measurable objectives; one testable hypothesis (if the study compares groups).
5 key references	From your PubMed search in Session 2 (literature review worksheet / literature matrix), stored in Zotero or another reference manager; Vancouver style; at least 3 from the last 10 years.

Part 2 — Built in Sessions 3–4 (Design; Bias and effect)

Section	What we expect
Study design & justification	Name the design and say why it fits your question (and why a stronger design is or is not possible).
Setting	Type of unit/hospital, period of the study (no real hospital name needed).
Population	Inclusion and exclusion criteria, written so two people would select the same patients.
Exposure/intervention & comparator	What exactly is given or measured, and what it is compared with.
Outcomes	One primary outcome (with how and when it is measured) + 1–3 secondary outcomes.
Main biases & how they are reduced	At least 2 specific threats (selection, information, confounding) and the step that reduces each (randomisation, blinding, standard definitions, matching, adjustment...).

Part 3 — Built in Sessions 5–6 (Data; Analyse)

Section	What we expect
Variables table	Outcome, exposure and 3–6 confounders/other variables, each with its type (binary, categorical, ordinal, continuous) and unit.
Mini-CRF & data dictionary	A data collection form of 6–10 items, and matching data-dictionary rows (name, definition, type, unit/codes, allowed range).
Data management & quality plan	How data will be captured (paper CRF or electronic data capture, e.g. REDCap or KoBoToolbox), coded, checked (range and consistency checks, double entry or spot checks), stored securely with backups, and who can change them (audit trail).
Sample size statement	One or two sentences: expected outcome rates (from the literature), the difference you want to detect, power, significance level and the resulting number — or, for a descriptive study, the precision you want. Show the tool you used.
Analysis plan in plain language	How each variable will be summarised (mean/SD, median/IQR, n/%), which test or model for the primary comparison, which confounders you will adjust for, and how you will avoid multiple-comparison problems (one pre-specified primary outcome).
Missing-data plan	How you will prevent missing data, how you will report it (how much, for which variables), and what you will do about it (e.g. complete-case analysis, plus multiple imputation as a sensitivity analysis if a statistician is available).

Part 4 — Built in Session 7 (Protect and write)

Section	What we expect
Ethics & consent	Is it research, audit or QI? Ethics committee approval route, who consents and how (including patients who lack capacity, if relevant), risks and benefits.
Registration	Where and when the study will be registered (e.g. ClinicalTrials.gov or a WHO ICTRP primary registry for trials, before the first participant; PROSPERO for systematic reviews), or why registration is not required.
Confidentiality	De-identification (study codes), who holds the key, where data are stored, who can access them, how long they are kept.
Feasibility	Timeline (simple Gantt chart), budget headings (no figures needed), people and resources, expected recruitment per month and how you know it is achievable.
Publication plan — target journal & reporting guideline	One target journal (checked with Think. Check. Submit.; scope, open-access fee) and a back-up journal; the reporting guideline for your design (CONSORT, STROBE, STARD, PRISMA, SQUIRE...), found via EQUATOR.
Publication plan — authorship & CRediT roles	Who will be an author (ICMJE criteria), in what order, and each person's CRediT role (e.g. conceptualisation, methodology, data curation, formal analysis, writing – original draft); who will be acknowledged instead.
Publication plan — working title	The final working title (from Part 1, refined).
Publication plan — structured-abstract skeleton	About 200 words: Background (1–2 sentences), Methods (design, setting, participants, primary outcome, analysis), <i>expected</i> Results (what you will report, with placeholders, e.g. "28-day mortality was X% vs Y% (RR ..., 95% CI ...)") — no invented numbers), Conclusion (what each possible result would mean).
Dissemination	How results will be fed back to the unit (meeting, poster, conference abstract) and to patients if relevant.

Length: about 6–8 pages, including the mini-CRF, Gantt chart and abstract skeleton (appendices are fine). Short and clear beats long.

Which session builds what (proposal-builder slots):

Session	Proposal builder	Part
S1	Topic, PICO, research question, FINER check, aim and objectives	1
S2	Search string, 5 references in the literature matrix, problem statement, working title	1
S3	Design and justification, setting, population (inclusion/exclusion)	2
S4	Exposure and comparator, outcomes, main biases and mitigation	2
S5	Variables table, mini-CRF, data-dictionary rows, data-management plan	3
S6	Sample size, analysis plan, missing-data plan	3
S7	Ethics and consent, registration, confidentiality, feasibility, publication plan	4
S8	Presentation (no new writing in class)	—

3. Group roles

Agree roles in Session 1. Everyone writes; the roles make sure the work actually gets done. In a group of five, combine Timekeeper with Scribe. The same group also writes the manuscript ([Manuscript_Brief.pdf](#)), which has its own roles; one person can hold one proposal role and one manuscript role.

Role	Responsibilities
Facilitator	Keeps discussion on the task, makes sure every voice is heard (including the quieter professions), summarises decisions.
Scribe	Types into the shared proposal document during group work; keeps the single "master" version; tidies formatting.
Timekeeper	Watches the clock in proposal-builder sessions; calls "5 minutes left"; times presentation rehearsals at 6 minutes.
Presenter	Leads the 6-minute presentation in Session 8 (others may present one slide each); prepares the 6 slides with the team.
Literature lead	Runs the PubMed search, keeps the 5 key references in a reference manager (e.g. Zotero) and the literature review worksheet; leads the publication plan (target journal, reporting guideline).
Methods lead	Owns design, outcomes, variables, sample size and analysis plan; checks Parts 2 and 3 are consistent with each other.

Roles may rotate between sessions if the group prefers, but the presenter should be fixed by the end of Session 6.

4. Deadlines

When	What is due
End of Session 1 (Week 1, Saturday)	Topic agreed; PICO, research question, FINER check, aim and objectives drafted.
End of Session 2 (Week 2, Saturday)	Part 1 complete: problem statement, working title and 5 key references.
End of Session 4 (Week 4, Saturday)	Part 2 complete. Finish any gaps before Session 5 (Week 5).
End of Session 6 (Week 6, Saturday)	Part 3 complete. Homework before Session 7 (Week 7): start the 6 slides (Presentation_Guide.pdf) and think about the target journal and authors.
End of Session 7 (Week 7, Saturday)	Part 4 complete (ethics, registration, feasibility, publication plan). Homework during the week: finish the 6 slides.
Before Session 8 starts	Slides sent to the course lead (one file per group, GroupN_slides.pptx or PDF) so all presentations run from one laptop.
Session 8 (Week 8, Saturday)	Presentation of the proposal and publication plan (6 min + 2 min feedback).
2 weeks after Session 8	Final written proposal (Parts 1–4, revised after the feedback) submitted for written feedback — the same deadline as the manuscript response letter.

5. Submission

- **Format:** one Word file per group, using the template, named [GroupN_Proposal.docx](#) (e.g. [Group3_Proposal.docx](#)). PDF is also accepted.
- **Where:** by email to the course lead, or to the shared course folder announced in Session 1.
- **Include on the cover page:** group number, working title, names and professions of all members, and each member's group role. (Authorship and CRediT roles go in the publication plan, Part 4.)
- **Use of AI tools:** you may use AI tools to improve language or to brainstorm, but say so in a short statement at the end ("AI use: ..."), check every fact and reference yourself, and never let AI invent references. The group is responsible for every word (Session 7, segment 7.2).
- **Confidentiality:** do not include any real patient data, names or record numbers. Numbers you quote (e.g. "about 40 admissions with septic shock per year") should be approximate unit figures, not extracted records.
- **Originality:** write in your own words; cite every source. Copying text from papers or the internet without quotation marks and a citation is plagiarism.

6. Feedback

- **Presentation (Session 8):** each group receives 2 minutes of verbal feedback from instructors and one peer group, plus a completed feedback form (see [Proposal_Rubric.pdf](#)).
- **Written proposal:** marked with the rubric in [Proposal_Rubric.pdf](#) and returned with written comments within about 3 weeks of submission.

- **The marks are formative** — they are there to help you improve the proposal, not to pass or fail anyone.

Groups are expected to share the work fairly; if a member did not contribute, the facilitator should tell the course lead privately.

After the course: strong proposals can be developed into a full protocol and statistical analysis plan (templates in [Course/Templates/](#)), registered and taken to your research or ethics office. When the data are collected, the manuscript skills from Sessions 7–8 apply directly. This course is **Level 1**; **Level 2** (*Doing the analysis*, [Course_Levels/level2_hands_on.md](#)) teaches you to run the analysis yourself in R, hands-on, and **Level 3** ([Course_Levels/level3_advanced.md](#)) offers advanced methods for complex data as workshops or consultancy.